



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : اسماء محمد عبد الحميد عبد الوهاب
المحافظة : الفيوم الحى / المدينة : سنورس

رقم البطاقة : 29512140100544 رقم التليفون : 01203137761

العمر (تاريخ الميلاد) : 14-12-1995 المهنة : ربه منزل

العنوان : التوفيقيه
تاريخ تسجيل الإستمارة : 10-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي :
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغه [11127]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: اسماء محمد عبد الحميد عبد الوهاب National ID: 29512140100544	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
قبالة ←
زوجة ←



MDT FORM

Name:	Asmaa Mohamed Abdelhamid .	MRN:	712
Age:	31 Y	ID:	
Menopausal Sta	PRE MENOPAUSAL	Phone:	010238 8769
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-VE	Reason Of MDT:	SURGERY

Clinical Data:

BX:

ER: PR: HER2: KI67:

R

L

Sonomammography:

Mets Workup:

History:

received 4 AC + 4 taxotere + zoladex --> clinically responding ended 28/2/2024
LT MRM (after PT counselling PT refused re construction)

IDC GII WITH FOCAL DCIS NON COMEDO 15% AND POSITIVE PERINEURAL
INVASION
FREE SKIN
YPT1(1CM) YPN1(2/10) WITH NO EXTRACAPSLUR EXTENSION
PARTIAL PATHOLOGICAL RESPONSE
FREE DEEP MARGINS

5/2024 ADJUVANT :
1/PORTH
2/AI + ZOLADEX

7/2026 MRI ABD AND PELVIS :
right ovarian cystic lesion.

multi loculated right ovarian cystic lesion
eliciting homogenous low T1S1, high T2S1. The lesion is seen measuring about
10.5 x10.5. x8 cm in maximum CC, TS & AP dimensions, showing thin internal
septations
demonstrating mid post contrast enhancement. No enhancing mural nodules or
solid soft tissue component noted within.

TUMOR MARKER :CA 125 +AFP+CA15-3 ,,,NORMAL

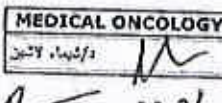
Recommendations:

FOR SURGICAL COSULTATION

MDT Outcome & Plan

For pt counseling on surgical resection vs short term follow up

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name: <u>اسماء محمد عبد الحميد</u>		
MR. : <u>712</u>		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها		
Breast cancer on Femara + Zoladex; Zoladex next month. Left breast UIQ with LNs, cTm N2 M0, ER 8/8 PR 5/8, HER2 0, received NACT, MRM PPR stage ypT1b ypN1, PORT, AI + Zoladex today for AI + supportive for 3 M; next Zol 28/5 pharmacy 5/5/2026 APUS – large right adnexal multilocular cystic lesion showing multiple septations and basal thick septum likely originating from right ovary – for further post-contrast pelvic MRI. Femara + supportive *3 m today: Zoladex next clinic visit: 3/9/2026. Femara + supportive *3 m 7/2026 MRI pelvis: right ovarian cystic lesion 10 × 10 for tumor marker + surgical consultation next visit: 5/10/2026.		
تحريراً في ,		
التاريخ :	21-09-2026	الوقت :
الطبيب المعالج :	نهى سعيد علي محمد	
يعتمد ,		

Patient Name Asmaa Mohamed Abdel Hamed	Date 05 / 05 / 2026
Exam : Ultrasound ABDOMEN & PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

The liver

Is average in size measures 15cm at Right MCL and showing homogenous echo pattern.
No definite focal lesion is seen. No intrahepatic biliary dilatation is noted.
The Portal vein is patent.

The gallbladder normal shape with No Stones or mud , The common bile duct is normal in caliber.

The pancreas No gross mass.

The spleen is average in size measures 10 m, homogenous echo pattern, No focal lesions.

Both kidneys are normal in size and shape and echogenicity, normal parenchymal thickness and CM difference.

No stones or back pressure changes on both sides.

No dilated pelvicalyceal system,

The urinary bladder is adequately filled, no mass or stones.

Uterus and adnexa

large right adnexal multiloculated cystic lesion showing multiple internal septations and basal thick septum likely originating from the right ovary.

Uterus , normal size and shape , normal endometrial thickness , no myometrial masses seen.

Normal shape of left ovary

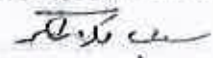
No ascites is noted.

OPINION:

Right adnexal/ ovarian complex cystic lesion for further pelvic post contrast MRI assessment.

Colonic gaseous distention

Best regards
Dr. Mahmoud Ashour
Dr Abu Bakr Hejazy FEPR MSc



Patient Name: ASMA MOHAMED ABD ELHAMED	Patient ID: 29512140100544
Report Date: 05/07/2026 02:45:20 PM	Age: 30 Years, 6 Months, 23 Days

DYNAMIC MRI STUDY OF THE ABDOMEN AND PELVIS

Technique of examination:

- Axial T1 (in-out phases) & T2 WIs .
- Coronal & sagittal T2 WIs .
- Post contrast axial T1 WIs.
- Axial DWI & ADC

Findings:

- The hallmark of the study is the presence of a multi loculated right ovarian cystic lesion eliciting homogenous low T1SI, high T2SI. The lesion is seen measuring about 10.5 x 10.5 x 8 cm in maximum CC, TS & AP dimensions, showing thin internal septations demonstrating mild post contrast enhancement. No enhancing mural nodules or solid soft tissue component noted within. No diffusion restriction. No hemorrhage or fat content .
- Enlarged size of the liver (right lobe span= 17.5cm). No focal hepatic lesions, areas of abnormal enhancement or dilatation of the intra hepatic-biliary radicals.
- Patent homogeneously opacified portal vein and its main branches.
- Normal size & MR signal of the spleen, with no focal lesions within its parenchyma.
- Partial distention of the gall bladder with no filling defects seen inside.
- Normal site, size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, IVC, both supra-renals & urinary bladder
- No ascites.
- No pathologically enlarged pelvi-abdominal lymph nodes.
- Normal MRI appearance of the rest of the pelvic organ.
- Normal MRI appearance of the lung bases.

Conclusion:

- ❖ *right ovarian cystic lesion as described above , for gynecological assessment.*

Dr. Eman Sayed

Dr/zeinab mostafa,MD

حليوز ولسر



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: اسماء محمد عبدالحليم عبد الوهاب

Sex:

MDT form

3- Medical number:
Female

83161923

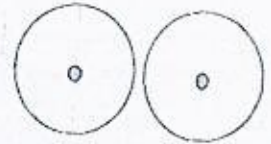
Old Med:

Old Med

3395-2023

2-ID number: 29512140100544

Patient Age: 28 Y



Family history

Family History
-VE

Phone

Phone

01201317761

BMI

BMI

Marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co Morbidity
NAD

Menopausal
status

Menopausal Status
PERIMENOPAUSAL

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT
POST OPERATIVE

Date of MDT

Date of MDT

02/05/2024

Breast Imaging :

Clinical data

Mets Workup :

few scattered bilateral pulmonary
indeterminate nodules for follow up
bilateral adnexal cystic

BX

IDC

ER

8/8

PR

5/8

Her 2 Neu

0

SISH

Ki67

30

T

2M

N

2

M

0

Sonomammography

left breast UIQ multiple suspicious masses with ipsilateral pathological axillary L.N BIRADS 6

PATIENT HISTORY

received 4 AC + 4 taxotere + zoladex --> clinically responding ended 28/2/2024
LT MRM (after PT counselling PT refused re construction)

IDC GII WITH FOCAL DCIS NON COMEDO 15% AND POSITIVE PERINEURAL INVASION
FREE SKIN

YPT1(1CM) YPN1(2/10) WITH NO EXTRACAPSLUR EXTENSION
PARTIAL PATHOLOGICAL RESPONSE

FREE DEEP MARGINS

RCB : 3.168 , RCB CLASS II

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

1- ADJUVANT ZOLADEX FEMARA (ABEMACILIB FROM NGO)
2- PORT

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

د/أحمد جابر

SIGNATURE

RADIOLOGY

RADIOLOGY

د/محمد النجدي

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

MDT Form

[[DOCTYPE:MDT_FORM]]

Patient Name : الأستاذ / اسماء محمد عبد الحميد
Patient No. : 29512140100544
Path No. : 1213/24
Sex : Female
Age : 28 Y
Referring Doc :

Request No. : 13008350
Request Date : 01/04/2024
Verification Date : 07/04/2024
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 23x17x7 cm with covering skin ellipse measured 22x15 cm.
- * Tumor extent: An ill defined greyish white firm tumor bed site.
- * Size: 6 x4 x3 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 1.3 cm away from deep margin.
- * Lymph nodes number and size: 10 L.Ns up to 1.5x1 cm.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+) Grade II.
- * Maximal tumor diameter 1 cm
- * Actual tumor cellularity: 17%, tumor necrosis index: 83%
- * Ductal carcinoma in situ: Focal DCIS (non comedo) 15%.
- * Perineural invasion: Positive.
- * Lymphovascular emboli: Occasional.
- * Skin involvement: Free skin.
- * Lymph node involvement: Positive in (2/10) with no extracapsular extension.
- * Resection Margins: Free deep margin.
- * Histologic response to therapy: Partial pathological response.
- * Microcalcifications: No.
- * Additional pathologic findings: No.

Verified by :
DR.Menna Nabil

Printing Date : 17/04/2024 11:48:00

Page 1 of 2

Printing By : marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Asmaa Mohamed Abd Al Hamid Abdel Wahab	Date : 06/09/2023
Exam : C.T CHEST	Age : 27 Year(s)
Medical ID: 339/2023	41045

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast UIQ few soft tissue masses the largest measuring about 1.9 x 1.7 cm.
- Left level I axillary pathologically enlarged lymph nodes, the largest measuring about 2.6 : 2.2 cm.
- Few scattered bilateral pulmonary nodules are noted the largest noted in the lower right lung lobe 8 mm, few left basal and lingular atelectasis.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bilateral adnexal cystic lesions are noted the largest noted in the right side 3.8 x 4.6 cm.
- No abdominal or pelvic masses or collections.



Patient Name: Asmaa Mohamed Abd Al Harnid Abdel Wahab	Date : 06/09/2023
Exam : C.T CHEST	Age : 27 Year(s)
Medical ID: 339/2023	41045

➤ No sizable osseous lytic or sclerotic lesions in the scanned skeleton.

Opinion:

- *Left breast multiple soft tissue masses associated with ipsilateral axillary lymphadenopathy.*
- *Few scattered bilateral pulmonary indeterminate nodules, for follow up.*
- *Bilateral adnexal cystic lesions, for dedicated US assessment.*

Dr. Mohammed Mahjoub

Dr. Reham Anwar, FEBR, MD

MUHAMMED, ASMAA Patient ID: 123025904 DOB: 14-Dec-1995 Date: 04-Dec-2023
Patient Lastname: Muhammed
Patient Firstname: Asmaa
HIS Patient ID: 123025904
RIS Patient ID: 343854
Date of Birth: 14/12/1995

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (4/12/2023)
*

- * blood pool Anterior and Posterior images for 1000k count followed by Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:
- " Homogenous radiotracer distribution is noted all over the surveyed skeleton.
- " Both kidneys are seen.
- Conclusion: -
- " The skeleton is keeping free from metastatic osseous deposits and is considered normal for age.

Much Obligated
Alaa Elsaeed
Nahla Mamoun, MSc

===== REPORT TEXT ENDE =====